
The Year of Your Neck and Shoulders

The upper body deep cut — what's actually wrong
when these areas hurt, and what to do about it

NATE RATCLIFF, LMT

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HEALING HANDS BY NATE

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Inner Circle Deep Cut — #2

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Introduction

AN INNER CIRCLE DEEP CUT. YEAR-LONG COMPANION for the most common upper-body complaints.

If the lower back is the most common complaint that walks into my practice, the neck and shoulders are a very close second. After 12+ years on the table, I'd estimate they account for another 30-40% of what brings people in.

Like the lower back, the neck and shoulders get blamed for problems that often originate elsewhere. People come in saying "my neck is killing me" when what's actually happening is mid-back tightness, jaw clenching, or shoulder forward-roll from desk work. The neck just happens to be where the symptom shows up loudest.

The neck is loud because it's small, complex, and full of nerves. Small problems create big sensations. That's a feature, not a bug — your neck is trying to tell you something. The trick is reading the signal correctly.

This is the deep cut on the upper body — what the neck and shoulders actually are, the patterns I see most often, how to read your own signals, what to do at home, and when to escalate to me or to a doctor.

Reference this all year. The upper body has seasonal patterns just like the low back does, and you'll want different sections at different times.

CHAPTER 1

How Your Neck and Shoulders Actually Work

THE “NECK AND SHOULDERS” IS A REGIONAL SYSTEM, not two separate parts. They share muscles, share innervation, share movement patterns. When one is in trouble, the other usually is too.

The cervical spine (your neck)

Seven vertebrae from the base of your skull to where your neck meets your shoulders. The cervical spine has the highest range of motion of any region of your spine — it has to, because it positions your head for vision, hearing, and balance.

The first two vertebrae (C1 and C2) are specialized for head rotation. C1 (the atlas) holds up your skull. C2 (the axis) provides the pivot. Most of your head-turning motion happens at this joint.

The lower five cervical vertebrae (C3-C7) provide most of the flexion (chin to chest) and extension (looking up). They’re also where most cervical disc issues happen.

Between each pair of vertebrae are facet joints (small joints that guide movement) and discs (the shock absorbers). When the neck is mobile and the surrounding tissue is healthy, all of this works smoothly. When it's not, you get pain.

The shoulder girdle

This is where it gets complex. What people call “the shoulder” is actually a system of four joints working together:

Glenohumeral joint — the ball-and-socket where your upper arm bone meets your shoulder blade. The most mobile joint in your body. Also the least stable, which is why shoulder dislocations are common.

Scapulothoracic joint — not a true joint, but the relationship between your shoulder blade and your rib cage. The shoulder blade should glide smoothly across the rib cage as your arm moves.

Acromioclavicular (AC) joint — where your collarbone meets the top of your shoulder blade. Often involved in injuries from falling on the shoulder.

Sternoclavicular joint — where your collarbone meets your sternum (chest bone). Rarely a problem, but worth knowing about.

For the shoulder to work right, all four joints have to coordinate. If one is restricted, the others compensate — and compensation patterns are where most shoulder pain comes from.

The muscles connecting neck and shoulders

This is the key — neck and shoulders share their primary muscles:

Upper trapezius — runs from the base of your skull and cervical spine down to the top of your shoulder. When tight, both neck and shoulder feel it.

Levator scapulae — runs from upper cervical vertebrae to the top inner corner of your shoulder blade. The “I have a knot under my shoulder blade that won’t go away” muscle.

Suboccipitals — small deep muscles at the base of your skull. When tight, they cause tension headaches and reduce neck rotation.

Pectoralis minor — small muscle in the front of your chest. When tight (which it is in most desk workers), pulls the shoulder forward and indirectly tightens the upper back trying to compensate.

Sternocleidomastoid (SCM) — runs from behind your ear down to your collarbone and sternum. Stress muscle, posture muscle, often a hidden contributor to headaches.

Scalenes — three small neck muscles. Stress breathing involves overusing these. They can also compress nerves to the arm when tight (thoracic outlet syndrome).

You don't need to memorize this. What matters is the principle: neck and shoulder muscles are interconnected. Treating one without the other is incomplete.

CHAPTER 2

The Five Upper-Body Patterns I See Every Week

LIKE THE LOW BACK, THE UPPER BODY HAS PREDICTABLE patterns. After thousands of sessions, these are the five I see most often.

Pattern 1 — Desk Worker Hunch

Symptom: Neck and shoulders progressively tighter through the workday. Shoulders feel “up around the ears” by 3 PM. Headaches by evening, especially behind the eyes or at the temples.

What’s happening: Hours of arms-forward, head-forward posture. Pec minor shortens, pulling shoulders into rounded forward position. Upper traps and levator scapulae overwork to hold the head up against the forward pull. Suboccipitals chronically tight from forward head position.

Who gets it: Anyone who works at a desk, looks at a phone a lot, drives a lot — most modern adults.

What helps: Doorframe stretch (non-negotiable), the 20-minute rule from Module 6, open-book stretch from Module 8, chest-opener exercises.

On the table: I work the pecs and front of the chest, the upper traps, the levator scapulae, the suboccipitals, often the thoracic spine. The neck “knots” can’t fully release until the chest opens.

Pattern 2 — Tension Storage

Symptom: Constant background tension in the neck and shoulders regardless of activity level. Worse during stressful periods. Often accompanied by jaw tension.

What’s happening: The neck and shoulders are common stress-storage sites. Chronic sympathetic nervous system activation keeps these muscles in low-grade contraction. The chronic tension isn’t from physical work — it’s from the body bracing against perceived threat.

Who gets it: People in high-stress life seasons. Caregivers, parents of young children, people in financial or relationship strain.

What helps: Stress-interruption techniques (vagus nerve work from Module 4, breathing from Module 2), regular bodywork, addressing the underlying stress where possible.

On the table: I do tissue release but also work specifically with breath and parasympathetic activation. The tissue won’t stay released if the nervous system stays activated.

Pattern 3 — Sleep Position Damage

Symptom: Neck pain that's worse in the morning. Sometimes specifically one-sided. May have woken up with a "crick" in the neck that took days to resolve.

What's happening: Pillow too thick or too thin, sleeping in a position that puts cervical spine out of alignment, sleeping on stomach (which forces neck rotation for 7-8 hours).

Who gets it: Anyone with the wrong bedding setup. Stomach sleepers especially.

What helps: Check the pillow against the criteria in Module 3 (head in line with spine when side-sleeping). Switch from stomach sleeping if that's your pattern. Sometimes a cervical pillow with neck support.

On the table: I can release the morning stiffness, but if the sleep position is wrong, it'll be back the next morning. The fix has to happen in the bedroom.

Pattern 4 — Rotator Cuff Issues

Symptom: Sharp pain in the shoulder during specific movements, especially overhead reaching. Pain reaching behind your back. Pain sleeping on the affected side.

What's happening: The rotator cuff — four small muscles that stabilize the shoulder joint — is irritated, strained, or impinged. Causes include sudden injury,

repetitive overhead work, or compensation patterns from a tight mid-back.

Who gets it: People who do overhead work (painters, contractors, athletes), people with chronically poor shoulder mobility, people over 50 (rotator cuff tissue weakens with age).

What helps: Pendulum swings (from Bonus 10 — Hunting protocol), rotator cuff strengthening exercises (look up “rotator cuff Y, T, W exercises”), addressing the mid-back to reduce compensation.

On the table: I can work the rotator cuff and surrounding tissue. But significant tears need orthopedic evaluation, and some rotator cuff problems require physical therapy or surgery.

When to see a doctor: Sudden severe shoulder pain after a specific injury, inability to lift the arm at all, persistent night pain not relieved by changing position.

Pattern 5 — Frozen Shoulder / Adhesive Capsulitis

Symptom: Progressive loss of shoulder range of motion over weeks or months. Pain at first, then mostly stiffness. Inability to reach behind back or overhead. Often one-sided.

What’s happening: The capsule around the shoulder joint thickens and tightens, restricting motion. Cause is often unclear — sometimes after a minor

injury, sometimes after a period of disuse, often without obvious trigger.

Who gets it: Most common in adults 40-60, especially women, especially those with diabetes or thyroid issues.

What helps: Patience. Frozen shoulder is a 12-18 month condition that resolves on its own in most cases. Physical therapy can help maintain whatever range of motion remains. Pushing too hard makes it worse.

On the table: I work gently within the available range, focus on the surrounding compensation patterns (neck, mid-back, opposite shoulder taking extra load), and provide pain relief. But I cannot “force” the capsule to release — that’s the trap many practitioners fall into.

Critical: Frozen shoulder has three phases — freezing (painful), frozen (stiff), thawing (regaining motion). The treatment is different in each phase. Get an orthopedic evaluation if you suspect frozen shoulder.

CHAPTER 3

Self-Assessment

USE THIS TO IDENTIFY WHICH PATTERN (OR combination) is yours.

Question 1 — When does it hurt most?

- Progressively through the workday → Pattern 1
- During stressful life periods → Pattern 2
- Mornings, when waking → Pattern 3
- Specific movements (overhead, behind back) → Pattern 4
- All the time but mostly stiff, getting worse over weeks → Pattern 5

Question 2 — Where exactly?

- Both upper traps, both sides of neck → Pattern 1 or 2
- One-sided neck and shoulder → Pattern 3 or 4
- Deep in the shoulder joint → Pattern 4 or 5
- Base of skull, into temples (headaches) → Pattern 1 or 2 with suboccipital involvement

Question 3 — Have you lost range of motion?

- No, just pain → Likely Pattern 1, 2, 3, or 4 (early)
- Yes, progressive loss over weeks → Possible Pattern 5 — get evaluated
- Yes, but only one specific movement (overhead) → Likely Pattern 4

Question 4 — Does anything cause sharp pain?

- Sharp pain only with specific arm movements → Pattern 4 or 5
- Dull constant ache or background tension → Pattern 1, 2, or 3
- Sharp pain at rest, especially at night → See doctor

Question 5 — Does the pain refer anywhere?

- Stays in neck/shoulder area → Likely muscular
 - Refers down the arm (especially with numbness/tingling) → Possible nerve involvement — see doctor
 - Refers into headache pattern → Likely muscular/postural
-

CHAPTER 4

The Daily Upper-Body Maintenance Routine (10 Minutes)

DAILY CARE FOR THE NECK AND SHOULDERS. Designed for general maintenance, not therapy for acute issues.

Morning (3 minutes)

- Neck rotations, slow, 5 each direction
- Ear-to-shoulder tilts, 15 seconds each side
- Shoulder rolls, 10 each direction
- Doorframe stretch, 30 seconds

Midday (2 minutes)

If you sit/work at a desk, every 90 minutes:

- Stand up
- Doorframe stretch, 30 seconds
- Cross-body shoulder stretch, 30 seconds each side
- Neck rotation check — am I locked up?

Evening (5 minutes)

Before bed:

- Open-book stretch, 5 reps each side
- Doorframe stretch, 30 seconds × 3
- Slow neck circles, both directions
- Cross-body shoulder stretch, 30 seconds each side

The doorframe stretch is the non-negotiable. If you do nothing else daily, do that one.

CHAPTER 5

Seasonal Upper-Body Issues

THE NECK AND SHOULDERS HAVE PREDICTABLE seasonal patterns.

Spring (March-May)

Primary issue: Tax season for desk workers — long hours, hunched posture, stress storage. Plus the start of yard work activating the shoulder girdle that's been dormant.

Prevention: Daily maintenance routine. Pre-yard-work warm-up that includes shoulder mobility.

Summer (June-August)

Primary issue: Air-conditioning-body cycle (see Bonus 6). Cold AC plus warm outdoors = constant adjustment that tightens neck and shoulders.

Prevention: The Bonus 6 protocol. Cover your neck in cold environments. End-of-day heat application.

Fall (September-November)

Primary issue: Cumulative summer load showing up. Plus hunting season for those who hunt (see Bonus 10).

Prevention: Fall recovery protocol (Module 10). Pre-hunt warm-up.

Winter (December-February)

Primary issue: Cold-weather stiffness (Module 11). Plus holiday stress storage in upper traps.

Prevention: Daily morning warm-up. Hot shower protocol. Stress-interruption techniques.

CHAPTER 6

Tools, Props, and Products That Actually Help

Worth buying

LACROSSE BALL (\$5): THE SINGLE BEST TOOL for upper trap release. Place against a wall and lean into it on the tight spots. Better than most “trigger point” devices.

Foam roller (\$20-40): Use it on the mid-back to indirectly help the neck and shoulders (mid-back stiffness causes neck issues).

Rice sock (homemade): See Bonus 2. Outstanding for neck and shoulder tightness, especially before bed.

Cervical pillow (varies): If you have specific cervical issues and a regular pillow doesn’t work for you, a cervical pillow with proper neck support can help. Try before buying — not everyone needs one.

Pec stretching strap (cheap version is just a towel): For aggressive chest opening if the doorframe stretch isn’t enough.

Probably worth it for some people

Theracane or backnobber (\$20-30): Long J-shaped tool for self-release of upper traps and rotator cuff areas you can't easily reach. Useful for some people, gimmicky for others.

Massage gun (\$100-300): General relief. Doesn't solve underlying patterns. Don't use directly on the neck (especially the front of the neck — too many critical structures).

Don't waste your money on

Neck traction devices: Most cervical traction devices don't do what they claim. The science is mixed. Save your money unless your physician specifically recommends one.

"Posture corrector" braces: Like back braces, these often weaken the muscles they're supposed to support. The muscles get dependent on the brace.

Vibrating neck pillows / massagers: Feel nice. Don't address underlying issues. Often expensive for what they do.

CHAPTER 7

When to See Me, When to See a Doctor, When to Wait

Wait (try home care first)

- Generalized neck/shoulder tightness from a known cause (long drive, hard day at desk)
- Mild morning stiffness that resolves with movement
- Pattern you've had before that has resolved before

Come see me

- Tightness lasting more than 1-2 weeks despite home care
- Recurrent patterns
- Pre-event or post-event for big physical demands
- Sleep disruption from neck/shoulder discomfort
- You've identified your pattern (1-4 above) and want hands-on work

See your doctor

- Sudden severe shoulder pain after specific injury

- Numbness or tingling down the arm or into the hand
- Weakness in the arm (can't grip, can't lift)
- Loss of bowel/bladder control or coordination — go to ER immediately (rare but serious)
- Frozen shoulder pattern (progressive loss of range over weeks)
- Pain that wakes you up regularly and doesn't relieve with position change

Coordinated care

COMPLEX SHOULDER ISSUES OFTEN BENEFIT from:

- Physician for diagnosis and rule-out
 - Orthopedist for joint-specific evaluation
 - Physical therapist for graded rehabilitation
 - Bodyworker (me) for tissue release and pattern work
-

CHAPTER 8

Stories From the Table

“I keep getting the same knot — can you just dig it out?”

PEOPLE COME IN POINTING TO A SPECIFIC SPOT under their shoulder blade or in their upper trap. They want me to “dig it out.” I work the area. They feel better for a few days. The knot comes back. They want to dig it out again.

The knot isn’t the problem. The knot is the response to something upstream — usually a postural pattern, sometimes stress, sometimes a sleep position. Digging out the knot temporarily relieves it but doesn’t address why the knot keeps forming.

The fix: address the upstream pattern. The knot stops forming.

“It’s just my pillow — I’ll get a new pillow”

People assume their morning neck pain is the pillow’s fault. Sometimes it is. Often it’s also the position they sleep in, the screen time before bed, the jaw clenching

during the night, and the daytime forward-head posture all contributing.

The fix: yes, fix the pillow if it's wrong. AND address the other factors.

"I just don't have time to stretch every day"

Five minutes. That's the daily maintenance routine. Less time than you spent waiting for coffee this morning.

The reality: "no time" usually means "no priority." When neck and shoulder pain gets bad enough, people find the time. Then it resolves and they stop. Then it comes back.

The fix: just make it a default before pain forces the issue. The maintenance is cheaper than the rescue.

CHAPTER 9

What to Expect in a Year of Consistent Work

MONTH 1-2: AWARENESS OF PATTERNS YOU didn't notice before. Some immediate relief from acute issues.

Month 3-4: Daily routine becomes automatic. Morning stiffness reduces. Headache frequency drops.

Month 5-6: Baseline shifts. Less catching yourself "shoulders up around ears." Easier to take full breaths. Better range of motion in neck rotation.

Month 7-12: Continued refinement. You know your patterns. You know your triggers. The neck and shoulders become a much smaller part of your daily body awareness.

CHAPTER 10

Closing

THE NECK AND SHOULDERS ARE LOUD BECAUSE they're delicate — full of nerves, full of fine-motor muscles, positioned to demand your attention when something's wrong. That loudness can be useful if you listen. It can be miserable if you don't.

This document is a reference. Come back when something specific is happening. Use the patterns to figure out what. Use the routines to address it. Use the “when to see whom” guidance to escalate appropriately.

The companion workbook has a pattern self-assessment, a 30-day upper-body tracker, and three reflection prompts to help you map your own neck and shoulder story over the year.

— Nate



This book was crafted by Nate Ratcliff,
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